جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : احسان امبابئ احمد حرب
المحافظة : الجيزه **الحي / المدينة :** العياط **العنوان :** -
رقم البطاقة : 28211292100422 **رقم التليفون :** 01121519383 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 29-11-1982 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: احسان امبابئ احمد حرب
National ID: 28211292100422

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Zoledronic acid for bone protection during hormonal therapy**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



٢٨٢١١٢٩٢١٠٠٤٢٢ ٢٠٢٤/٠٨
ربة منزل
أنثى مسلمة متزوجة
عويس سلامه احمد حرب
١ البطاقة سارية حتى ٢٠٣١/٠٨/١٢



جمهورية مصر العربية
بطاقة تحقيق الشخصية
احسان
امبابى احمد حرب
كفر عمار
مركز العياط - الجيزه
٢٨٢ ١١ ٢٩ ٢١ ٠٠٤ ٢٢
KR8151837

DMS
1141

فازلودكس + ايبرفس
زولادكس + زوميتا

National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

mm/dd/yyyy

MDT FORM

Name:	Ihsan Imbabi Ahmed .	**MRN:**	1121
Age:	43 Y	**ID:**	
Menopausal Sta	PRE	**Phone:**	01121519388
NRS SCORE:	PS1	**Marital Status:**	MARRIED
BMI:		**Family History:**	NO
CO Morbidity:	HTN	**Reason Of MDT:**	

Clinical Data:

BX:

ER: +VE **PR:** +VE **HER2:** -VE **KI67:**

Sonomammography:

Mets Workup:
CT CAP (Apr 2026): PD with ? sclerotic bone mets.
Bone scan: PD with new lesions at sternum, Lt humeral head, Lt SI joint, distal Rt femur.

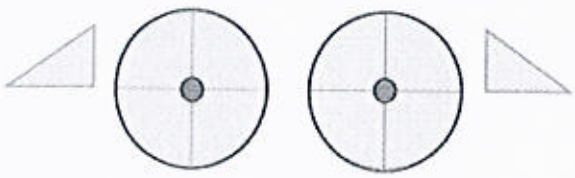
History:
CT4 N2 M1 to bone ,

started zoladex , femara , zometa / 3 months (from 06/2025) and no FU was requested SINCE THEN

CT CAP (Apr 2026): PD with ? sclerotic bone mets.
Bone scan 04/ 2026 : PD with new lesions at sternum, Lt humeral head, Lt SI joint, distal Rt femur.

FOR YOUR EVALUATION AS ADVICED REGARDING : FASLODEX , IBRANCE , ZOLADEX ,
ZOMETA /3 MONTH

R L



TNM

Recommendations:

MDT Outcome & Plan
For :FASLODEX , IBRANCE , ZOLADEX
ZOMETA /3 MONTH

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ شري حسن	د/ عبد السلام شلبي	د/ ثروت رمضان			



MDT Form
[[DOCTYPE:MDT_FORM]]

Name: احسان امبابي احمد **MR. :** 1121	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

42 y old female , HTN, ER , PR +ve , HER 2 negative , CT4 N2 M1 to bone , started zoladex , femara , zometa / 3 months (from 06/2025) and no FU was requested zometa ,zoladex due date 22/04/2026 new assesement requested for evaluation of ttt and for femara

CT CAP (Apr 2026): PD with ← sclerotic bone mets. Bone scan: PD with new lesions at sternum, Lt humeral head, Lt SI joint, distal Rt femur. ZOLADEX + FEMARA TODAY MDT >> R

For - FASLODEX , IBRANCE , ZOLADEX ZOMETA /3 MONTH TODAY FOR C1D1 FASLODEX C1 IBRANCE ZOLADEX -- NEXT DUE ON 9/8 C1 D15 FASLODEX PHARMACY LABS OK

For - FASLODEX , IBRANCE , ZOMETA /3 MONTH TODAY FOR: C1D1 FASLODEX , C1 IBRANCE NEXT DUE ON 9/8 >>>> 21/7/2026 >>> C1 D15 FASLODEX + zoladex + zometa after labs >>> pharmacy

Ibrance + faslodex + zoladex + zometa >>> day care pharmacy labs 10/8/2026 >>> creat : 1.1 , ca ionized 1.02

C3 Ibrance Faslodex labs accepted , new evaluation CT CAP requested to be done after finishing this cycle

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : علياء احمد أبو النصر شهاب الدين

يعتمد ,

ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / احسان امبابي احمد حرب	Request No.	: 13006156
Patient No.	: 28211292100422	Request Date	: 22/01/2025
Path No.	: 303/25	Verification Date	: 12/02/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast multicentric malignant masses reaching nipple and skin, with right axillary tail mass.
Nature of Specimen: Tru-cut biopsy, collected at 21/01/2025, 12:37 pm.

Gross Examination:
1- Right para-areolar mass:
- Multiple grayish white tissue cores up to 1.2 cm, totally submitted.
2- Right axillary tail mass:
- Three grayish white tissue cores up to 0.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
1- Right para-areolar mass:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (2+2+2) Grade II.
* Ductal carcinoma in situ: DCIS (solid and cribriform pattern).
* Perineural invasion: Not detected.
* Angioinvasion: Not detected.
* Microcalcifications: Not detected.
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 85% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- Her2: No membrane reaction of tumor cells (score 0).
- Ki67: 20% nuclear reaction of tumor cells.
2- Right axillary tail mass:
* Histological type: Desmoplastic cores showing Invasive duct carcinoma.

Verified by :
Dr.Toka Medhat

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / احسان امبابي احمد حرب	Request No.	: 13006156
Patient No.	: 28211292100422	Request Date	: 22/01/2025
Path No.	: 303/25	Verification Date	: 12/02/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 42 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ghonaim
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

11 C1

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient's Name:	**EHSAN EMABABY AHMED HARB**
Date:	**2026-04-06**
Referred by:	Prof.Dr. CT CAP IV
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported right breast carcinoma , compared to last study dated on 28/3/2025

(I) Chest:

- **Right breast regression of size of spiculated mass 2.3 x2.3 cm compared 2.2 x 6 cm , attached to skin with retraction .**
- **Stationary course of enlarged right axillary lymph odes , the largest measures 19 mm.**
- **Clear both lung fields .no sizable pulmonary masses or consolidation patches**
- **No pleural or pericardial collections seen.**
- **No gross cardiac abnormality.**

(II) Abdomen and pelvis:

- **The liver is enlarged in size showing homogenous CT density , no focal lesions**
- **Patent and homogenously enhancing portal vein.**
- **No intra or extra hepatic biliary radicles dilatation**

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Patient's Name:	**EHSAN EMABABY AHMED HARB**
Date:	**2026-04-06**
Referred by:	Prof.Dr. CT CAP IV
Hospital:	**Goustave Roussey International Egypt**

- **Normal CT appearance of the pancreas, both kidneys, aorta and IVC.**
- **Normal CT appearance of the spleen .**
- **Regular filling of the urinary bladder showing no stones, masses .**
- **Unremarkable other pelvic organs.**
- **No retro-crural, para-aortic or pelvic lymphadenopathy.**
- **No ascites.**
- **D7, D8, L8 and left iliac bone sclerotic lesions with D7 structural collapse .**

OPINION

- **Regressive course of right breast mass**
- **Progressive course of spread of sclerotic bony lesion for correlation with bone scan .**
- **D7 collapse .**

Dr.Reham Khedr, MD

CT Scan
[[DOCTYPE:CT_SCAN]]

مركز النيل

SCAN & LABS

Name : Ihsan Emabie Ahmed Harb
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 18-03-2026 07:32
Medical No. : 334013
Ins.Company : تعاقد مستشفيات

ISOTOPIC BONE SCAN REPORT

- **Indication:** 43-year-old female with history of breast carcinoma, presenting with bone aches. Evaluation for possible osseous metastatic disease.
- **Prior Study (02/06/2025):** Active osseous lesions at DV7, left 7th rib posteriorly, and mid right humeral shaft; consistent with few metastatic osseous lesions (prior images not provided for comparison).
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection of the tracer (Tc-99m MDP).

FINDINGS:

Compared to the prior study, there is mild progression of metastatic bone disease:

- **Previously documented lesions (persistent):** DV7, left 7th rib posteriorly, mid right humeral shaft.
- **New lesions:** Sternum, left humeral head, left sacroiliac region (lower end), distal shaft of the right femur.
- **Incidental findings:** Age-related degenerative/arthritic changes in the spine and weight-bearing joints.

IMPRESSION:

- **Mild progression of metastatic bone disease** with new lesions identified in the sternum, left humeral head, left sacroiliac region, and distal right femur.
- Prior osseous lesions remain active.
- **For clinical correlation and follow-up as indicated.**

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Medical No. : 10191
Patient's Name: ehsan embaby ahmed
Date: 2026-09-26
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

جوستاف روسي انترناشونال مصر

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE NECK, CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION REVEALED:

Known case of breast CA.

I- NECK:

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- Multiple subscentemetric bilateral upper deep cervical and submandibular LNs are noted; likely non-specific.
- No pathologically enlarged cervical lymph nodes.

II- CHEST:

- Right breast lesion measuring 33 x 20 mm with few right axillary lymph nodes largest measuring 12 x 12 mm for sono-mammography.
- Tiny left apical calcified pulmonary nodule.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

III- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الأشعة

Patient Name: Ehanan Embaby Ahmed	Date : 28/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 42 Year(s)
Medical ID: 716/2025	Pat. ID: 98635

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- **Reported case of right breast cancer.**

I Chest:

- Both breasts are partially out of examination field.
- Right breast UIQ irregular skin attached mass lesion measuring 2.2 X 6 cm with focally thickened skin.
- Enlarged right axillary level I and II lymph nodes reaching 1.9 cm.
- Few right upper lung lobe nodules reaching 3mm.
- No sizable pulmonary masses, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Average sized liver showing homogenous parenchymal attenaution. No sizable focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Right Ischial sclerotic area.
- D7 lytic destructive lesion with suspected intraspinal extra-osseous soft tissue component.
- Few left humeral head lucent foci.

Opinion:

Reported case of right breast cancer revealed:

- **Few right pulmonary nodules mostly non specific for follow up.**
- **D7 lytic lesion mostly metastatic.**
- **Right Ishial and left humeral lesions for bone scan.**
- **No sizable hepatic lesions.**

Dr. Mohamed Mohsen, M.D.

الممسوحة ضوئيا بـ CamScanner

CT Scan
[[DOCTYPE:CT_SCAN]]

Medical No. : 10191
Patient's Name: ehsan embaby ahmed
Date: 2026-09-26
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple scattered axial and appendicular skeleton sclerotic bone lesions.

Opinion:

- **Tiny left apical calcified pulmonary nodule likely inflammatory/ granulomatous.**
- **Multiple sclerotic axial and appendicular skeleton bone lesions for bone scan.**
- **No pulmonary or hepatic metastatic lesions.**

Kindly arrange for
Comparison with last 4/2026

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]